Brigatinib monotherapy · ALK-positive advanced non-small-cell lung cancer (NSCLC) Brigatinib on its own for adults with ALK-positive advanced non-small-cell lung cancer: either after crizotinib (00562a) or before any ALK inhibitor (00562b). This is not an adjuvant course or a general pathway after every ALK inhibitor.

NATIONAL CANCER CONTROL PROGRAMME (NCCP) 00562 · VERSION 4

Brigatinib Therapy

The usual starting plan is 90 mg once daily for seven days, then 180 mg once daily if your team confirms the increase. Some people need a different dose. Treatment continues while helping and tolerable, with regular reviews and no fixed number of cycles.

Make treatment fit into daily life

Swallow the tablets whole with water, with or without food. Do not crush, chew or split them. Avoid grapefruit and grapefruit juice. If a dose is missed or you vomit after it, do not take an extra dose; take the next dose at its usual time. Contact the team for ongoing vomiting or difficulty keeping fluids or medicine down.

Prescriptions and supply

Check tablet strengths and the next supply before running out. Keep one written lead-in, increase and review plan.

Home treatment

Swallow the tablets whole with water, with or without food. Do not crush, chew or split them. Avoid grapefruit and grapefruit juice. If a dose is missed or you vomit after it, do not take an extra dose; take the next dose at its usual time. Contact the team for ongoing vomiting or difficulty keeping fluids or medicine down.

Visits and blood tests

Before starting: ALK result, blood count, kidney/liver tests, breathing-function assessment, electrocardiogram (ECG), blood pressure, fasting glucose, muscle enzyme creatine phosphokinase (CPK) and pancreatic enzymes. Review breathing symptoms on days 8 and 15, then monthly or sooner if needed. ECG and blood pressure are checked every two weeks initially, then monthly. Liver tests are every two weeks for the first three months, then as advised. Blood count, kidneys, fasting glucose, CPK and pancreatic enzymes are usually checked monthly. Scans follow your Consultant’s plan.

After a break

Do not restart or increase brigatinib yourself. A break of 14 days or more for a reason other than a side effect normally needs a new seven-day lead-in before the previously tolerated dose. A break because of a side effect follows a different recovery plan. Early lung inflammation can prevent the usual increase to 180 mg.

Contacts and daily life

Keep daytime and out-of-hours numbers available. Agree who books and reviews tests, and ask for support with travel, work or daily tasks.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00562 v4 · official regimen
- EMA Alunbrig product information and patient leaflet, current retrieved snapshot
- NCCP systemic anticancer therapy (SACT) Antiemetic Guidance V6 (2025), oral Tables 5 and 8
- NCCP NCIS Medical Oncology antiemetics V8 (2025), parenteral scope
- ALTA-1L final analysis: brigatinib versus crizotinib, 2021
- ALTA final analysis: crizotinib-refractory ALK-positive NSCLC, 2022
- HSE cancer treatment side effects and urgent advice

Source reconciled; independent clinical, pharmacy and patient/PPI review pending